(Updated January 2026) Detailed Population of Focus Definitions

The Populations of Focus definitions given below supersede the definitions originally described in the CalAIM Proposal of January 2021. Each Population of Focus includes a detailed definition for the Population of Focus with eligibility criteria; examples of eligible Members; and operational guidance to support implementation. MCPs must use the eligibility criteria provided to evaluate Members and **may not impose additional eligibility requirements for authorization of ECM**. For example, an MCP may not require that the ECM provider have a certain number of contacts with the Member as a condition of authorization. In the Populations of Focus definitions, "**adult**" is defined as an individual who is 21 years of age or older, and a "**child or youth**" is defined as an individual under 21. Consequently, the Children and Youth- specific definitions for ECM apply up to age 21, with limited exceptions as called out below for youth with a history of involvement with the child welfare system. When a Member under 21 is served in ECM and does not meet the adult ECM criteria upon turning 21, the Member should not be disenrolled from ECM due to aging out alone; rather, the ECM Provider and MCP should apply the MCP's usual, DHCS-approved "graduation" criteria to determine when the Member is ready to disenroll.

1. Individuals and Families Experiencing Homelessness

Individuals and families experiencing homelessness are among the highest-need and most vulnerable individuals in Medi-Cal, in that they lack access to shelter and food, both of which are critical to health. These individuals often have extensive medical and behavioral health needs that are difficult to manage due to the social factors that influence their health. This can result in reduced quality of life and high utilization of avoidable, costly services in EDs and inpatient settings that could be avoided with more timely and appropriate care management and potentially the provision of Community Supports. ECM provides the needed link between physical and behavioral health care and connection to housing and other resources associated with Social Drivers of Health (SDoH). In addition, communities of color are disproportionately impacted by homelessness in California and nationally, making ECM for this Population of Focus a critical tool in achieving racial health equity.

A. *(Updated January 2026)* Population of Focus Eligibility Criteria:

This Population of Focus went live for homeless adults and families with the launch of ECM in 2022. Eligibility criteria was broadened to include unaccompanied children and youth, as well as whole families, with the launch of ECM for children and youth in July 2023, as laid out below.[7]

Eligibility Criteria Applicable from January 2026 and Onwards

a. Adults (whether or not they have dependent children/youth living with them) who:

(1) Are experiencing homelessness, defined as meeting one or more of the following conditions[8]:

(i) Lacking a fixed, regular, and adequate nighttime residence;

(ii) Having a primary residence that is a public or private place not designed for or ordinarily used as a regular sleeping accommodation for human beings, including a car, park, abandoned building, bus or train station, airport, or camping ground;

(iii) Living in a supervised publicly or privately operated shelter, designed to provide temporary living arrangements (including hotels and motels paid for by federal, state, or local government programs for low income individuals or by charitable organizations, congregate shelters, and transitional housing);

(iv) Exiting an institution into homelessness (regardless of length of stay in the institution);

(v) Will imminently lose housing in next 30 days;

(vi) Fleeing domestic violence, dating violence, sexual assault, stalking, and other dangerous, traumatic, or life-threatening conditions relating to such violence;

[7] From 2022 until June 30, 2023, children and youth were eligible only as part of a family unit with a qualifying adult meeting the Individuals Experiencing Homelessness POF definition.

[8] This definition of homelessness is based on the U.S. Department of Health and Human Services (HHS) 42 CFR § 11302 - General definition of homeless individual with the modification to Clause (v) timeframe for an individual who will imminently lose housing has been extended from 14 days (HHS definition) to 30 days. The wording of this definition has also been slightly modified for clarity, relative to the originally released definition.

AND

(2) Have at least one complex physical, behavioral, or developmental need, with inability to successfully self-manage, for whom coordination of services would likely result in improved health outcomes **and/or** decreased utilization of high- cost services.[9]

OR

(3) Members authorized for Transitional Rent[10]

No further criteria are required to be met to qualify for this ECM Population of Focus. MCPs may not impose additional eligibility requirements for authorization of ECM.

As of July 2023, the ECM eligibility criteria for this Population of Focus also includes children and youth, as shown below. The purpose of these modifications is to ensure that ECM captures the breadth of unsafe, substandard, and insecure living conditions that families, children, and youth may experience. To do this, the Population of Focus eligibility criteria are modified for children and youth from 45 CFR 11431a McKinney-Vento Homeless Assistance Act's definition of "at risk of homelessness." The California Department of Education (CDE) uses a similar definition of homelessness to identify students in need of housing support. DHCS continues to strengthen partnerships with CDE, such as the requirement for MCPs and Local Educational Agencies (LEAs) to enter into memoranda of understanding (MOUs) per the upcoming Medi-Cal Managed Care contract procurement going live in January 2024.

b. Homeless Families or Unaccompanied Children/Youth Experiencing

[9] Pregnant and postpartum individuals who are homeless are considered to have met this definition.
[10] Refer to the Community Supports Policy Guide (Volume II) for further Transitional Rent policy guidance.

Homelessness

(In Effect July 1, 2023, and Onwards)

Children, Youth, and Families with members under 21 years of age who:

(1) Are experiencing homelessness, as defined above in (a) under the modified HHS 42 CFR Section 11302 "Homeless" definition;

OR

(2) Sharing the housing of other persons (i.e., couch surfing) due to loss of housing, economic hardship, or a similar reason; are living in motels, hotels, trailer parks, or camping grounds due to the lack of alternative adequate accommodations; are living in emergency or transitional shelters; or abandoned in hospitals (in hospital without a safe place to be discharged to to), as modified from the 45 CFR 11434a McKinney-Vento Homeless Assistance Act definition of "at risk of homelessness" [11]

OR

(3) Members authorized for Transitional Rent [12]

Notes on the (b) Definition:

- Children, youth, and families do not need to meet the additional "complex physical, behavioral, or developmental need" criteria noted above in Clause (2) for adults in (a).
- Clause (2) for children, youth, and families in (b) is modified from the 45 CFR 11434a McKinney-Vento Homeless Assistance Act definition of "at risk of homelessness"[13] and is included in this Population of Focus to ensure ECM captures the breadth of unsafe, substandard, and insecure living conditions that Members, particularly children and youth, may experience.

[11] See McKinney-Vento Homeless Assistance Act

[12] Refer to the Community Supports Policy Guide (Volume II) for further Transitional Rent policy guidance.

[13] See McKinney-Vento Homeless Assistance Act

No further criteria are required to be met to qualify for this ECM Population of Focus. MCPs may not impose additional eligibility requirements for authorization of ECM.

B. *(Updated January 2026)* Examples of Eligible MCP Members Under this Population of Focus:

- Member over age 21 experiencing homelessness who has complex health care needs as a result of unmanaged medical, psychiatric, or SUD-related conditions.
- Member with complex health care needs as a result of medical, psychiatric or SUD-related condition, who have recently received an eviction notice and will imminently lose housing in the next 30 days.
- Youth who has been excluded from home due to their gender identity or sexual orientation and is now temporarily living with the family of a friend (e.g., couch surfing).
- Parent and child who are fleeing domestic violence from a spouse at home.
- A pregnant person or person within 12 months postpartum experiencing homelessness.

C. Operational Guidance:

Identification:

- As for all ECM Populations of Focus, MCPs must establish policies and procedures that allow network providers to refer patients to the MCP for ECM if they suspect eligibility criteria are met and track key performance indicators to monitor and improve the volume of provider referrals.
- Engagement in ECM for this population may include street outreach or coordinating with shelters, Homeless Services Providers, Recuperative Care Providers, Community Partners (e.g., Homeless Coordinated Entry Systems) and other service Providers with experience working with homeless individuals and families.
    - Members and their families may self-refer to ECM.
    - MCPs are expected to utilize data from their regional Continuum of Care Homeless Management Information System (HMIS) to

determine which of their members are experiencing homelessness and should be referred for ECM.

(Updated January 2026) **Outreach and Engagement:**

- As is true for all ECM Lead Care Managers, ECM requires engagement with the Member in the community or at Provider locations. Street medicine provides an opportunity to provide care to individuals who are experiencing homelessness by meeting them where they are. Separately from ECM, DHCS issued a Street Medicine All Plan letter 24-001 to clarify how street medicine providers may utilize Presumptive Eligibility in mobile clinics, street teams or other locations to be able to provide immediate access to Medi-Cal Services. Street medicine providers may be ideally suited to conduct outreach and engage with Members who are experiencing homelessness, whether serving as ECM Lead Care Managers within their own teams or handing off to other ECM Providers who will take on the longitudinal role.
- MCPs are strongly encouraged to conduct outreach for eligible ECM Members within family homeless shelters and coordinate with homeless outreach teams and other entities where families are known to be experiencing or at risk of homelessness. Examples of these entities may include local housing and homelessness programs serving families or pregnant or postpartum people, Head Start, schools, local WIC agencies, diaper banks, food banks, county First 5s, and CBOs supporting pregnant and postpartum people or families experiencing intimate partner violence.

Comprehensive Assessment and Care Management Plan:

- For this Population of Focus, the Comprehensive Assessment and Care Management Plan should specifically address housing needs, in addition to any medical and/or social needs identified, and appropriate interventions to address these for the ECM Lead Care Manager to help connect the Member to sources of housing support, including (but not limited to) the pre-approved Community Supports. MCPs should ensure that ECM Providers serving members of this Population of Focus have detailed and up to date information about how to refer Members to housing Community Supports, including eligibility criteria and the operation of lifetime limits, where applicable.
- Per federal requirements, if the Member has LTSS needs, the care plan must be developed by an individual who is trained in person-centered planning, using a

person-centered process;[14] should consider and reflect what is important to the Member regarding their preferences for the delivery of LTSS (for example, specific treatment goals, services or functional needs the Member prefers to prioritize).

(Updated January 2026) **Examples of Applicable ECM Services for this Population of Focus:**

For this Population of Focus, ECM should include addressing barriers to housing stability by connecting Members and their families to housing, health and social support resources. Examples of applicable services for this Population of Focus to be included in ECM include (but are not limited to):

- Facilitating access to housing-related Community Supports to identify housing and preparing individuals to secure and/or maintain stable housing, including housing that meets the unique needs of pregnant and postpartum individuals and families.
- Maintaining regular contact with Members to ensure there are not gaps in the activities designed to address an individual's health and social service needs, and swiftly addressing any gaps to ensure progress toward regaining health and function.
- Coordinating and collaborating with various health and social services providers, including Regional Centers and county First 5s,[15] including sharing data (as appropriate) to facilitate better-coordinated whole-person care.
- Supporting Member treatment adherence, including scheduling appointments, appointment reminders, coordinating transportation, ensuring connection to public benefits, identifying barriers to adherence and accompanying Members to appointments, as needed.
- Utilizing best practices such as Housing First, Harm Reduction, Progressive Engagement, Motivational Interviewing, and Trauma Informed Care.

(Updated January 2026) **Community Supports:** MCPs are strongly encouraged to offer the housing- related Community Supports to Members who enroll in ECM under this

[14] As established in 42 CFR § 438.208 and 42 CFR § 441.301.

[15] **County First 5s** are local, county-based entities in each of California's 58 counties to provide essential resources and services at the local level to support child development, care, and education of children up to age five. Please visit the First 5 webpage for additional information.

Population of Focus.[16]

- Transitional Rent provides up to six months of rental assistance in interim and permanent settings. When the MCP authorizes a Member for Transitional Rent, it must also authorize the Member for ECM. The MCP is required to ensure that the ECM Provider conducts weekly in-person outreach visits to the Member.[17] Transition Navigation Services, Housing Deposits, and Housing Tenancy and Sustaining Services provide support to achieve long-term housing.
- (Housing Trio) Housing Transition Navigation Services, Housing Deposits, and Housing Tenancy and Sustaining Services provide support to achieve long-term housing.
- Recuperative Care (Medical Respite) and Short-Term Post-Hospitalization Housing provide recovery-focused housing.
- Medically Tailored Meals/Medically-Supportive Food, Day Habilitation Programs, or Sobering Centers may also address the needs of this Population of Focus, depending on individual circumstances.

(Updated January 2026) **Provider Contracting:**

- MCPs must contract with ECM Providers specializing in each of the specific Populations of Focus, who have an existing footprint in the communities they serve. As suggested above, MCPs are encouraged to contract with street medicine providers and other organizations specializing in serving individuals experiencing homelessness to serve as ECM Providers, provided they meet all other ECM Provider requirements. MCPs are also encouraged to contract with housing providers that meet the needs of individuals from pregnancy through 12 months postpartum, including with family shelters, to ensure the individual has a safe place to live during pregnancy and through 12 months postpartum. Please refer to Section VII. ECM Provider Network below for more information.
- For homeless families, MCPs are strongly encouraged to work with ECM Providers to serve the family unit together through one ECM team, whenever

[16] For the comprehensive menu of Community Supports services and their corresponding service definitions, refer to the Community Supports Policy Guide: Volume 1 (non-housing) and Volume II (housing).

[17] Refer to the Community Supports Policy Guide (Volume II) for further Transitional Rent policy guidance on automatic ECM authorization, guidance on ECM Provider outreach, and implementation timeline.

possible and appropriate. MCPs may develop specific payment models (e.g., bundled rates) to reimburse ECM Providers for ECM delivered to a family.

2. Individuals At Risk for Avoidable Hospital or ED Utilization

In 2022, DHCS renamed the "High Utilizers" ECM Population of Focus to "Individuals At Risk for Avoidable Hospital or ED Utilization," to reflect that ECM for this Population of Focus aims to reduce avoidable care in costly settings and settings that are of higher acuity than would be necessary with earlier and more whole- person care interventions and approaches.

A. Population of Focus Eligibility Criteria:

a. Adults At Risk for Avoidable Hospital or ED Utilization

Adults who meet one or more of the following conditions:

(1) **Five or more** emergency room visits in a **six-month** period that could have been avoided with appropriate outpatient care or improved treatment adherence;[18]

(2) **Three or more** unplanned hospital and/or short-term skilled nursing facility (SNF) stays in a six-month period that could have been avoided with appropriate outpatient care or improved treatment adherence.

No further criteria are required to be met to qualify for this ECM Population of Focus. MCPs may not impose additional eligibility requirements for authorization of ECM.

For this POF, MCPs may choose to authorize ECM for individuals who are at risk for avoidable hospital or ED utilization and who would benefit from ECM but who may not meet the numerical thresholds specified above. This flexibility is in addition to and does not displace the numerical thresholds provided in the eligibility criteria.

[18] Including appropriate timing, interventions within outpatient care settings, care plan development, communication with the Member, interdisciplinary care team, or referrals.

b. Children and Youth At Risk for Avoidable Hospital or ED Utilization

Children and youth who meet one or more of the following conditions:

(1) **Three or more** ED visits in a **12-month** period that could have been avoided with appropriate outpatient care or improved treatment adherence;[19]

(2) **Two or more** unplanned hospital and/or short-term SNF stays in a **12-month** period that could have been avoided with appropriate outpatient care or improved treatment adherence.

No further criteria are required to be met to qualify for this ECM Population of Focus. MCPs may not impose additional eligibility requirements for authorization of ECM.

For this POF, MCPs may choose to authorize ECM for individuals who are at risk for avoidable hospital or ED utilization and who would benefit from ECM but who may not meet the numerical thresholds specified above. This flexibility is in addition to and does not displace the numerical thresholds provided in the eligibility criteria.

B. Examples of Eligible MCP Members under this Population of Focus:

- An adult with repeated incidents of avoidable ED visits in a six-month period who has a medical, psychiatric or SUD-related condition requiring intensive coordination beyond telephonic intervention.
- An adult with repeated incidents of avoidable ED visits in a six-month period who has significant functional limitations and/or adverse SDOH that impede them from navigating their health care and other services.
- A child with repeated incidents of ED visits in a 12-month period to address untreated asthma that could have been avoided with improved inhaler and nebulizer treatment.

[19] Including appropriate timing, interventions within outpatient care settings, care plan development, communication with the Member, interdisciplinary care team, or referrals.